

COMMENT

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The professionalisation paradox: retaining the health advantages of community-based providers in task shifting and task sharing initiatives

Shukanto Das^{1*} , Liz Grant¹ and Aaron Orkin²

Abstract

Task shifting and task sharing involve the deliberate redistribution of healthcare responsibilities to non-specialist workers to maximise resources, expand access, and enhance health. Structured training, regulatory measures, and monitoring can transform non-specialist workers into nascent professionals. While this is often assumed to be beneficial, it can create new scarcities and resource pressures. We call this the “Professionalisation paradox”: task shifting and sharing initiatives are often successful because they bring non-professional workers and approaches into healthcare delivery, but the very success of those programmes frequently prompts the professionalisation of the workers and roles involved. We explore paramedicine as a case study, where emergency responders started as part of a task shifting-based approach from military medicine to fill gaps in civilian emergency care, but have now become a professional group bound by rules that can restrict their flexibility. Professionalisation offers advantages such as improving care standards, safety, and worker employment security. However, this also means that paramedics are less able to act freely and compassionately, as they might feel limited by regulations or fearful of making mistakes. We propose ways to balance the benefits of professionalisation while keeping task shifting and task sharing adaptable and community-focused. Future approaches to creating task shifting and task sharing-based models of service delivery might include combining structured training with more flexible methods. This will allow providers to use their skills without feeling overly restricted. Creating supportive policy frameworks around these roles could help maintain accountability and give them more room to respond to patient needs. Striking this balance will help ensure that task shifting and task sharing retain their core purpose of providing accessible, responsive care.

Keywords Task shifting, Task sharing, Professionalisation, Professional boundaries, Paramedicine, Emergency medical services, Community health workers, Human resources for healthcare, Health workforce regulation, Health policy

Commentary

Task shifting and task sharing (TS/S) are healthcare management strategies that redistribute appropriate clinical or non-clinical services among providers, with an aim to improve access to care, quality, cost efficiency, and equity in a system facing staff shortages [1]. Task shifting transfers roles from specialised providers to those with lesser training, while task sharing entails collaboration across people with varied responsibilities [2]. TS/S have been

*Correspondence:

Shukanto Das
shukanto.das@ed.ac.uk

¹ Usher Institute, The University of Edinburgh, 5-7 Little France Road, Edinburgh BioQuarter (Gate 3), Edinburgh EH16 4UX, UK

² Department of Family and Community Medicine, University of Toronto, 500 University Avenue, 5Th Floor, Toronto, ON M5G 1V7, Canada



mainstays in global health efforts, with applications spanning infectious diseases, mental health, maternal and child health, and non-communicable disease management across geographical contexts [3]. Contemporary frameworks recognise that TS/S happen across different cadres, for example, top-down redistribution from physicians to nurses or bottom-up initiatives where lay providers step into care roles to address community needs, each requiring unique implementation strategies, emphasising that TS/S leverage community connections and cultural competencies of providers [1, 2].

Successful TS/S, especially those involving non-professional workers, often prompt a movement to formalise those roles, which is a move commonly assumed to be unambiguously beneficial. However, this introduces a paradox. Efforts to professionalise TS/S via structured training and monitoring generate regulated semi-professionals, paraprofessionals, or entirely new professions. Throughout this commentary, we distinguish professionalisation as a sociological process from the application of clinical standards or evidence-based practice. Professionalisation refers to the formation of occupational groups that claim jurisdictional control over work, training, standards, and their relationships with the state and the market. Professionalisation is the dominant contemporary mechanism for safeguarding quality, safety, and accountability in modern healthcare systems, but tends also to generate provider scarcities and associated access inequities. Adherence to clinical standards and commitments to patient safety and quality can exist without necessarily requiring the full professionalisation of a workforce, and professionalisation need not be seen as the sole pathway to safety or quality. This standardisation can improve technical quality and safety. Still, it simultaneously alters the social organisation of care in ways that encroach on the responsiveness, accessibility, and community orientation that initially enabled TS/S to succeed [4]. The professionalisation paradox lies in the fact that while professionalisation strengthens providers by enhancing their skills and legitimacy, it can obstruct the core benefits that made TS/S onto lay providers effective in the first place, namely, their delivering care in community-centred, flexible ways that extend beyond professional boundaries. The professionalisation paradox is a paradox in the classical sense of an apparent or productive contradiction that arises from underlying assumptions and invites theoretical resolution, rather than an inherently irresolvable conflict. Importantly, the professionalisation paradox does not arise because standards or regulation are undesirable, but because the sociological consequences of professionalisation can undermine the access, equity, and relational strengths that TS/S was designed to preserve, even when technical quality

improves. The professionalisation paradox is most pronounced in bottom-up, community-embedded forms of TS/S involving lay providers, rather than top-down role redistribution among already-professionalised cadres such as physicians and nurses.

Professionalisation theory and TS/S

Professionalisation theory looks at how loosely defined roles evolve into regulated occupations. Professionalisation involves formal training, task standardisations, and regulations enforced by local or national governing authorities or, often, associations of self-governing practitioners. Professionalisation theory posits that as roles professionalise, they gain social recognition. Practitioner norms and standards are established, instituting notions of professional identity, intellectual monopoly, and defined career pathways. This gatekeeping results in occupational closures, where only people with specific qualifications can perform certain tasks, setting professionals apart from unqualified individuals and creating groups with status and power [5]. Professionalisation can create and entrench the very scarcities and inaccessibility that drive TS/S initiatives. TS/S is, in this sense, a disruptive redistributive process that reshapes professional boundaries and threatens professionals' primacy [1]. One solution is to launch non-professional providers toward professionalisation, enabling collaboration between existing professionals and new cadres [4]. However, examples from emergency medical services (EMS) described below show that professionalisation takes away from the intended cultural and relational connectedness.

Professionalisation paradox in emergency paramedicine

Military medicine has heavily influenced civilian EMS, but civilian-first models also shaped EMS. One iconic example is Pittsburgh's Freedom House ambulance service, which was founded in 1967. Faced with a system rife with racism intertwined with police response, marginalised Black residents faced severe barriers to accessing emergency care. They were organised to learn anatomy and physiology and trained in first aid and EMS. This prototypical TS/S strategy positioned locals as skilled paramedics with strong community ties. Their shared experiences brought compassionate responsiveness to work, improving quality and access. Freedom House ultimately established the foundation of American EMS [6]. A similar approach is seen with Hatzalah ambulances. Founded in Brooklyn in the 1960s, Hatzalah relies on thousands of trained volunteers within the Orthodox Jewish community, each equipped with medical kits in their private vehicles, for immediate intervention before ambulances arrive

and to assist patients in addressing their cultural needs within hospital systems [7]. By involving trusted community members in roles performed by formal EMS, Freedom House and Hatzalah show that community-focused TS/S satisfy clinical needs and cultural sensitivities while easing system pressures.

As EMS professionalised, protocols, regulations, and certification transformed paramedicine from a community-based group of trained lay responders to a specialised clinician class administering medications, interpreting electrocardiograms, and managing multi-organ complications. Paramedicine adopted an identity closer to conventional professions. While this improved technical reliability, paradoxically, it distanced EMS from the responsive community-oriented values at its core. For example, alongside formalisation, Freedom House faced political challenges and racial biases. Eventually, Pittsburgh created a centralised EMS, which led to Freedom House's closure in 1975. Many of the Freedom House paramedics drawn from the community were eliminated from the new service, while others were hired but given reduced non-clinical roles [6]. This demonstrates the professionalisation paradox in practice. Formalising EMS created structured systems and, eventually, national paramedical standards, but at the cost of excluding the very community members central to its impact. Freedom House's closure led to measurable declines in service quality for Black communities in Pittsburgh [6]. The compassionate responsiveness that defined its community-driven contributions was lost as collateral damage in a process of professionalisation. By prioritising professionalisation, systems risk sidelining the cultural knowledge, empathy and trust that less formalised community-based responders bring to care, which are key determinants in improving healthcare quality.

Hatzalah [7] continues to show that community-centred, task-shared EMS can thrive within regulated systems. Engaging culturally aligned volunteers with rapid response capabilities meets standards and community needs, showing that professionalisation can coexist with adaptable, community-driven models. Similarly, other modern EMS are exploring ways to retain this balance, as seen with London Ambulance's Community Responder Schemes, which train locals in first response before paramedics arrive [8]. Community-based Indigenous programmes in northern Canada have been similarly shown to meet the needs of communities, providing care that respects cultural values while improving access [9]. These examples show that, with intentional strategies, professionalisation does not necessarily have to come at the cost of community-centred care. Systems can retain the adaptability and cultural sensitivity that TS/S bring while benefiting from standardisation.

Rethinking professional bias

In the face of crushing health human resources shortages and inequities worldwide, TS/S is an essential ingredient in achieving more sustainable systems and delivering accessible care for all. The professionalisation paradox exposes the challenges inherent in achieving quality care without losing the community-based, flexible, and relational advantages of non-professional approaches. Systematisation, which extends across multiple TS/S contexts beyond EMS, transforms caregiving from informal, community-rooted practices into defined professions, increasing the professional bias. For example, research on community health workers reveals that by only prioritising certification requirements and not valuing their relationship-building capabilities and lived experiences, formalisation can distance providers from their communities, overshadowing what they do most effectively, which is advocating for localised positive changes [4]. Mental health peer support workers face similar tensions, where formalisation diminishes the uniqueness of services that operate separately from traditional mental health care, hindering authentic engagement that relies on shared experiences rather than credentials [10]. Traditional birth attendants represent a dramatic example of this bias, where policymakers, especially in low- and middle-income countries, focus on technical response capabilities perhaps without attending sufficiently to the accessibility and relational features of traditional attendants, creating pressures to eliminate traditional birthing practices regardless of their contributions [11]. In home-based care, interestingly, caregivers often exceed prescribed scopes, driven by the personal nature of their work, sometimes risking abuse and dissatisfaction. Naturally, caregivers feel torn between patient needs and professional boundaries [12]. This preference for professional structures over culturally grounded, community-based approaches indicates how professionalisation can undermine the qualities that make TS/S effective.

A contemporary example of TS/S without overt professionalisation is the Friendship Bench programme, which trains community grandmothers to deliver structured mental health support with supervision from psychiatrists [13]. Its effectiveness depends precisely on the non-professional social positioning, trust, and relational authority of these lay providers, rather than their incorporation into a new professional cadre. Friendship Bench trains and supervises lay community workers to deliver structured mental health interventions without establishing a new professional occupation, illustrating how TS/S can scale while avoiding full professionalisation. TS/S with deliberate but only partial professionalisation may also help to navigate the professionalisation paradox. Partial professionalisation trajectories are evident in

large-scale TS/S such as India's Accredited Social Health Activists [14] and Ethiopian Health Extension Workers [15], where lay, community-embedded roles gained training, standardisation, and supervision without becoming full professions. Instead of focusing solely on the benefits of formalised systems, a nuanced approach might recognise the strengths of informal care and find ways to champion them without seeking professionalisation. Community-based caregiving allows understanding of individual needs and personalities, something standardised systems find difficult to accommodate. For example, paramedics are expected to perform specialised roles efficiently, but can find themselves constrained by protocols that limit their ability to respond compassionately to unique situations. For instance, strict organisational policies, documentation requirements, and procedural protocols may prevent a paramedic from providing comfort measures they know would help. A fear of regulatory violations or litigation may cause paramedics to prioritise compliance over compassionate care, altering the nature of patient interactions [16]. Similarly, community health workers, peer support workers, and home care providers may feel constrained by these boundaries, even when patients need flexible, empathetic approaches. While valuing professional expertise, services must retain the relational strengths of community-based caregiving. This involves supporting caregivers with resources, legal protection, and structures, enabling them to balance standards with holistic, personalised care.

Recommendations and conclusion

In applying the professionalisation theory to EMS, we see how paramedicine has progressed from flexible TS/S to formalised professions. While this has brought technical consistency, it has distanced providers from community-centred roles. Governance frameworks should consider empowering providers to use skills and compassion without facing restrictive professional boundaries while upholding accountability, safety standards, and legal protections for providers. These frameworks might also establish flexible methods of competency-based training that allow for diverse learning pathways. This is important because newly professionalised roles typically involve shorter training than established professions; while longer educational programmes give practitioners the foundational knowledge to make judgments autonomously, shorter ones can increase dependence on rigid protocols, limiting flexibility and compassionate responsiveness. Ongoing, competency-based training alongside mentorship programmes pairing experienced community members with newly trained providers can help mitigate this disparity. Adaptive modules that preserve decision-making autonomy for culturally appropriate care

are essential. For example, training could incorporate community elders as co-facilitators, ensuring cultural knowledge remains central to professionalisation. Creating roles that combine professional competencies with community advocacy functions could also preserve the relational aspects that make TS/S effective. The magic of TS/S lies in blending this relational flexibility of lay care with the technical expertise of professionalisation. Therefore, collaborative systems leveraging the strengths of lay caregiving without defaulting to professionalisation cycles are key. Achieving this will require us to recognise the worth of lay contributions and support them rather than replacing them. The future of TS/S hinges on how we balance the need for structure and safety without losing the essence that makes TS/S valuable.

List of abbreviations

EMS	Emergency medical services
TS/S	Task shifting and task sharing

Acknowledgements

Not applicable.

Author contributions

All authors contributed to the development of the core idea presented in this paper. SD wrote the main manuscript text. LG and AO critically reviewed the manuscript and made edits. All authors reviewed and approved the final version of the manuscript.

Funding

No specific funding was received for this work.

Data availability

No datasets were generated or analysed during the current study.

Declarations

Ethics approval and consent to participate

Not applicable.

Consent for publication

Not applicable.

Competing interests

The authors declare no competing interests.

Received: 2 June 2025 Accepted: 19 February 2026

Published online: 12 March 2026

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